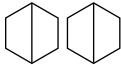


TERBRON SYRUP

TERBUTALINE SULPHATE SYRUP



DESCRIPTION

Clear, almost colorless, cherry-flavored solution.

COMPOSITION

Each 5 ml contains : **Terbutaline Sulphate 1.5 mg**

(Contains Methylparaben 0.1% w/v and Propylparaben 0.02% w/v as preservatives)

PHARMACODYNAMICS

Terbutaline is a selective β_2 -adrenoceptor agonists, thus producing relaxation of bronchial smooth muscle.

PHARMACOKINETICS

Absorption :

About 33-50% of an oral dose of terbutaline sulphate is absorbed from the GI tract. Following oral administration, a change in flow rate usually occurs within 30 minutes ; substantial clinical improvement in pulmonary function occurs within 1-2 hours and is maximal within 2-3 hours.

Duration of action following oral administration ranges from 4-8 hours.

Distribution :

Terbutaline sulphate is apparently distributed into milk in concentrations at least as high as concurrent plasma concentrations ; however, the amount distributed into milk is estimated to be less than 1% of a dose given to a nursing woman.

Elimination :

The sulphate conjugate is a major metabolite of terbutaline, and urinary excretion is the primary route of elimination. Excretion of the drug and its metabolites is essentially complete within 72-96 hours after a single oral dose.

INDICATIONS

For the relief of bronchospasm in bronchial asthma and bronchitis.

DOSAGE AND ADMINISTRATION

Adults :

1 tablespoonful, 2-3 times daily

Children :

below 3 years : 1/2 teaspoonful, 3 times daily

3-7 years : 1/2 - 1 teaspoonful, 3 times daily

7-15 years : 1-2 teaspoonfuls, 3 times daily

or as prescribed by the physician.

PREGNANCY AND LACTATION

Studies in animals have not revealed any teratogenic effects of terbutaline. There are, however, no adequate and well-controlled studies in pregnant women. Because animal reproduction studies are not always predictive of human response, terbutaline should be used during pregnancy only if the benefits clearly outweigh the risks. Terbutaline is secreted into breast milk, but effect on the infant is unlikely at therapeutic doses ; however, the drug should be used with caution in nursing women and only if the benefits outweigh the risks.

CONTRAINDICATIONS

Terbron is contraindicated in patients known to be hypersensitive to sympathomimetic amines or any components of the formulation. Should not be used in patients with thyrotoxicosis.

WARNING AND PRECAUTIONS

Tocolysis : Serious adverse reactions including death have been reported after administration of terbutaline/salbutamol to women in labor. In the mother, these include increased heart rate, transient hyperglycaemia, hypokalemia, cardiac arrhythmias, pulmonary edema and myocardial ischaemia. Increased fetal heart rate and neonatal hyperglycaemia may occur as a result of maternal administration.

As with all sympathomimetic, terbutaline should be used with caution in patients with cardiovascular disorders, including ischemic heart disease, hypertension, and cardiacarrhythmias ; hyperthyroidism and diabetes mellitus. Safety and effectiveness in children below the age of 12 have not been established.

ADVERSE EFFECTS

The frequency of side effects are an increase in heart rate, changes in blood pressure, nervousness, tremor, palpitation and dizziness. Headache, nausea, vomiting, anxiety, restlessness, lethargy, drowsiness, weakness, flushes, sweating, chest discomfort, muscle cramps and tinnitus have also been reported.

DRUG INTERACTIONS

- Sympathomimetic Agents

Terbutaline should not be administered concurrently with other sympathomimetic agents because of the possibility of additive adverse cardiovascular effect.

- Beta-Blockers

The action of terbutaline is antagonized by β -adrenergic blocking agents such as propranolol.

- Monoamine Oxidase Inhibitors and Tricyclic Antidepressants

Terbutaline should be administered with caution to patients being treated with monoamine oxidase inhibitor or tricyclic antidepressants, because the action of terbutaline on the vascular system may be potentiated.

OVERDOSAGE

There is no specific antidote. Treatment of terbutaline overdose is symptomatic and supportive therapy including cardiac and respiratory support. In the conscious patient, emesis should be induced followed by gastric lavage. In the unconscious patient, the airway should be secured with a cuffed endotracheal tube before lavage, and emesis should not be induced.

Administration of activated charcoal may be considered. The patient should remain under medical supervision until symptoms have resolved.

STORAGE CONDITION

Keep container tightly closed.

Store at temperatures not exceeding 30°C, protect from light and excessive heat.

SUPPLY

Bottle of 60 ml



Manufactured by
BIOLAB CO., LTD.
625 Bangpoo Industrial Estate,
Samutprakarn, Thailand.

Date of revision : 6 June 2019

EES 022/D

Note : #0=ไม่พบ #1=ไม่พบ Sulphate #2=ไม่พบ #3=ไม่พบ Description #4=ไม่พบ "tightly" (19/09/19)		Note : ☐ ถูกฉีกขาด ☒ RA ไม่พบ ☐ อื่น ๆ		Color Guide Black	
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